

BCG0100 – HIERARCHY OF GUIDANCE FOR ORGANIZATION DETERMINATIONS AND APPEALS



Benefit Type:	Medicare; Medi-Cal
Category:	Benefit Clarification Guideline
Effective Date:	5/28/2026
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Prior Authorization:	Prior Authorization May Be Required
Plan Info:	SCAN Plan Documents
Network Coverage:	In Network; Out of Network

OVERVIEW

This policy establishes SCAN Health Plan's framework for receiving, reviewing, deciding, and notifying members and providers of Organization Determinations and appeals, and defines the decision-making hierarchy used to ensure all medical necessity and coverage decisions are made consistently, accurately, and in accordance with Medicare Advantage requirements; it applies to all SCAN staff and delegated First Tier, Downstream, and Related Entities involved in these processes.

Note: Federal and state mandates, including Medicare statutes, state contract requirements, and DHCS Medi-Cal rules for dual-eligible members, may supersede all other criteria and must be applied before the Clinical Decision-Making Hierarchy.

POLICY/POSITION STATEMENT

Per CMS regulations, as a Medicare Advantage organization (MAO) SCAN must make medical necessity determinations based on all the following:

- Coverage and benefit criteria as specified at [§ 422.101\(b\) and \(c\)](#): SCAN may not deny coverage for basic benefits using coverage criteria not specified in § 422.101(b) or (c)
- Whether the provision of items or services is reasonable and necessary under the [Social Security Act §1862\(1\)](#)
- The member's unique medical history (for example, diagnoses, conditions, functional status), physician recommendations, and clinical notes
- Where appropriate, involvement of the organization's medical director as required at [§ 422.562\(a\)\(4\)](#).

Additionally, as outlined in [Medicare Managed Care Manual \(MMCM\) Chapter 4 Section 10.2 - Medicare Basic Rule](#), Medicare coverage and payment is contingent upon a determination that:

- A service is in a covered benefit category;
- A service is not specifically excluded from Medicare coverage by the Act; and
- The item or service is "reasonable and necessary" for the diagnosis or treatment of an illness or injury, to improve functioning of a malformed body member, or is a covered preventive service.

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Clinical Decision-Making Hierarchy for Organization Determinations and Appeals

In alignment with the Medicare Basic Rule, all SCAN providers, internal departments, and delegated entities must adhere to the following Clinical Decision-Making Hierarchy when considering whether services meet the reasonable and medically necessary standard under the [Social Security Act §1862](#) and [Medicare Basic Rule](#). This hierarchy *must* be applied in order, and no lower-level criteria may be used when higher-level criteria exist.

1. Centers for Medicare & Medicaid Services (CMS) Guidelines

CMS coverage requirements are the primary authority for all Organization Determinations and constitute Medicare's official coverage rules. They supersede all internal coverage criteria (non-CMS criteria) unless an applicable federal or state mandate requires otherwise. CMS guidance must be applied exactly as written. CMS sources include:

- Medicare National Coverage Determinations (NCDs)
- Medicare Local Coverage Determinations (LCDs)
- Medicare Local Coverage Articles (LCAs)
- Medicare Manuals, including Internet-Only Manuals (IOMs) (e.g., Medicare Managed Care Manual (MMCM))

2. Internal Coverage Criteria (Only When CMS Guidance Is Not Fully Established)

If CMS coverage guidance is incomplete, CMS allows SCAN to apply internal coverage criteria to determine whether treatment is reasonable and medically necessary based on current, publicly available, evidence-based clinical guidelines. SCAN's internal coverage criteria are posted on SCAN's website at [SCAN Health Plan Medical Policy](#). Internal Coverage Criteria is criteria outside of CMS guidelines and includes:

- SCAN Health Plan Medical Policies (based on current evidence in widely used treatment guidelines or clinical literature)
- Nationally Recognized Evidence-Based Guidelines: SCAN may use nationally recognized guidelines, applied in conjunction with the clinical judgment of a qualified health professional. Examples include:
 - Utilization Management Decision-Support Guidelines (e.g., MCG®, InterQual®)
 - Medical Journals or Publications, including but not limited to:
 - National Comprehensive Cancer Network (NCCN®)
 - American Diabetes Association (ADA®)
 - American Heart Association (AHA®)
- CMS LCDs or LCAs from Medicare Administrative Contractors (MACs)/Geographic Areas Outside of SCAN's Operating Areas
- Coverage Policies of Other MAOs: Consistent with MMCM Chapter 4, Section 90.5, SCAN may adopt the coverage policies of other MAOs in the same service area.

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Organization Determinations

SCAN must issue an Organization Determination for any request involving:

- Authorization of an item or service
- Payment for an item or service
- Continuation or reinstatement of previously authorized services
- Coverage of out-of-network services
- Reduction, suspension, or termination of previously authorized care
- Failure to provide timely authorization
- Requests for Continuity of Care during transitions between plans, provider organizations, or primary care providers

All adverse determinations must be reviewed by a physician or qualified health professional with appropriate expertise, consistent with 42 CFR §422.566(d).

Note: Adverse determinations using internal coverage criteria must first cite the relevant CMS criteria related to reasonable and medically necessary care (e.g., Medicare Managed Care Manual (MMCM) Chapter 4, Section 10.2, Basic Rule or Medicare Managed Care Manual (MMCM) Chapter 4, Section 10.16, Medical Necessity) prior to referencing the internal coverage criteria used (e.g., SCAN Medical Policy, MCG® or InterQual® Criteria, Medical Journal, Outside MAO Policy, or Out of Area LCD).

Continuity of Care Considerations

SCAN ensures continuity of care for members who are new to the plan, transitioning between provider organizations, or receiving ongoing treatment. SCAN collaborates with delegated Provider Organizations (POs) to ensure uninterrupted access to medically necessary services and safe transitions to in-network providers when appropriate. See BCG0150 – Continuity of Care for additional information.

Appeals (Reconsiderations)

- Members, their representatives, or providers acting on behalf of members may request a reconsideration of an adverse Organization Determination. SCAN must process appeals in accordance with:
 - 42 CFR §422.578–§422.592
 - Required timeframes for standard and expedited appeals
 - Automatic forwarding of upheld adverse decisions to the Independent Review Entity (IRE)

Documentation Requirements

All determinations must include:

- The specific criteria applied in the medical necessity determination, to include:

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- 1) Citation of CMS rule/guideline first (MMCM Chapter 4, Section 10.2 Basic Rule applies to all Medical Necessity Determinations)
 - 2) Any internal criteria used, if applicable (e.g., MCG, SCAN Medical Policy, etc.))
- Clear clinical rationale and supporting clinical evidence
 - Reviewer credentials
 - Alternate options if determination is adverse
 - Member and provider notifications consistent with CMS requirement

Delegation Requirements

Although SCAN Health Plan retains ultimate responsibility for compliance with Medicare Advantage regulations, all Delegated entities contracted with SCAN who perform UM or OD functions must:

- Follow the SCAN Clinical Decision-Making Hierarchy and documentation requirements outlined above
- Comply with all CMS, Federal, and State requirements
- Maintain documentation supporting each determination
- Participate in SCAN oversight and audits

DEFINITIONS

Adverse Organization Determination is a decision by SCAN or its delegated entity to deny, reduce, or terminate a requested service or to deny payment for a service.

Expedited Organization Determination is a request for a fast decision when applying the standard timeframe could seriously jeopardize the member's life, health, or ability to regain maximum function.

First Tier Entity is any party that enters into a written arrangement with SCAN, acceptable to CMS, to provide administrative or health care services to Medicare enrollees under the Medicare Advantage or Part D program.

Downstream Entity is any party that enters into a written arrangement with a First Tier Entity or another Downstream Entity to provide administrative or health care services related to the Medicare Advantage or Part D benefit.

Independent Review Entity (IRE) is the CMS-contracted entity that conducts external reviews of adverse reconsiderations.

Medical Necessity is a determination that services are reasonable and medically necessary for the diagnosis or treatment of illness or injury, consistent with Medicare coverage rules, as defined in [Social Security Act §1862\(1\)](#).

Medicare Basic Rule: Medicare coverage and payment are contingent upon a determination that a service is in a covered benefit category; is not specifically excluded from Medicare coverage by the Social Security Act; and the item or service is "reasonable and necessary" for the diagnosis or treatment of an illness or injury, to improve functioning of a malformed body

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member, or is a covered preventive service. These criteria are codified through rulemaking in the Code of Federal Regulations and/or applied in manual guidance or are applied through coverage determinations. (Source: [MCM Chapter 4, Section 10.2](#))

Related Entity is any entity related to SCAN by common ownership or control that performs management functions under contract or delegation, furnishes services to Medicare enrollees under an oral or written agreement, or leases real property or sells materials to SCAN at a cost exceeding \$2,500 during a contract period.

Organization Determination (OD) is SCAN's or its delegated entity's completed response to a request for authorization or payment of an item or service, including requests for continuation of previously authorized care or coverage of out-of-network services.

Out-of-Network (OON) Provider is a provider who is contracted with SCAN but not contracted with the member's assigned provider group, or a provider who is not contracted with SCAN at all.

Reconsideration (Appeal) is the member's or provider's request for SCAN to review an adverse Organization Determination.

REGULATORY CITATIONS/REFERENCES

- [Social Security Act §1862 - Exclusions from Coverage and Medicare As Secondary Payer](#) (Accessed 06/17/2026)
- [Medicare Managed Care Manual \(MMCM\) Chapter 4, § 10.2 - Medicare Basic Rule](#) (Accessed 06/17/2026)
- [Medicare Managed Care Manual \(MMCM\) Chapter 4, §10.16 - Medical Necessity](#) (Accessed 06/17/2026)
- [Medicare Managed Care Manual \(MMCM\) Chapter 4, Section 90.5](#) (Accessed 04/23/2026)
- [Medicare Managed Care Manual Chapter 4 §110.3](#) (Accessed 04/23/2026)
- [Medicare Program Integrity Manual Chapter 13 - Local Coverage Determinations](#) (Accessed 06/17/2026)
- [Code of Federal Regulations: 42 CFR 422.101 -- Requirements relating to basic benefits](#) (Accessed 06/17/2026)
- [Code of Federal Regulations: eCFR :: 42 CFR 422.562 -- General provisions](#) (Accessed 06/17/2026)
- [Code of Federal Regulations: 42 CFR §422.112\(b\)\(8\) Access to services](#) (Accessed 04/23/2026)
- [Code of Federal Regulations: 42 CFR §422.566 Organization determinations](#) (Accessed 04/23/2026)
- [Code of Federal Regulations: 42 CFR §422.578 Right to reconsideration](#) (Accessed 04/23/2026)

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- [Code of Federal Regulations: 42 CFR §422.580 Reconsideration defined](#) (Accessed 04/23/2026)
- [Code of Federal Regulations: 42 CFR §422.582 Request for standard reconsideration](#) (Accessed 04/23/2026)
- [Code of Federal Regulations: 42 CFR §422.584 Expediting certain reconsiderations](#) (Accessed 04/23/2026)
- [Code of Federal Regulations: 42 CFR §422.586 Opportunity to submit evidence](#) (Accessed 04/23/2026)
- [Code of Federal Regulations: 42 CFR §422.590 Timeframes and responsibilities for reconsiderations](#) (Accessed 04/23/2026)
- [Code of Federal Regulations: 42 CFR §422.592 Reconsideration by an independent entity](#) (Accessed 04/23/2026)
- [DEPARTMENT OF HEALTH & HUMAN SERVICES – Memo: Frequently Asked Questions related to Coverage Criteria and Utilization Management Requirements in CMS Final Rule \(CMS-4201-F\)](#) (Accessed 06/17/2026)
- [California Knox-Keene Health Care Service Plan Act §1373.96](#) (Accessed 04/23/2026)
- [Evidence of Coverage Documents | SCAN Health Plan](#) (Accessed 06/22/2026)

DISCLAIMERS

The information provided above is accurate as of the policy review date. However, CMS coverage policies are subject to periodic updates and revisions. As a result, this document may not reflect the most recent changes. For the latest information on applicable National Coverage Determinations (NCDs), Local Coverage Determinations (LCDs), Local Coverage Articles (LCAs), and CMS Online Manual System Transmittals, please visit the Medicare Coverage Database at: <https://www.cms.gov/medicare-coverage-database/search.aspx>.

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POLICY GOVERNANCE

Developed by: SCAN Medical Policy

Maintained by: SCAN Medical Policy

Impacted Teams: UM, Embrace, Medical Claims Review, Claims, Grievance and Appeals Department (GAD), Network Management, Medical Policy, Compliance

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POLICY HISTORY

Date:	Description:	Control #:
5/28/2026	UMC Approval	2-0100-05282026D
5/28/2026	Effective Date of Policy	2-0100-05282026F

RELATED POLICIES

BCG0150 – Continuity of Care

RETIRED/SUPERSEDED POLICIES

N/A